

Cancer therapies: the last six months

Clinical and preclinical publications, 9 March – 9 September 2026, across all therapeutic modalities

Literature synthesis of 18,424 curated records (7,671 clinical, 10,017 preclinical, 736 evidence syntheses)

Cancer therapies, clinical and preclinical, 2026-03-09 → 2026-09-09 — consolidated synthesis

A single end-to-end reference over the 18,424-record corpus indexed in `index.tsv`. It consolidates the modality notes (`notes/01`–`notes/09`) and adds cross-cutting analysis; methodology and caveats are in `notes/10`. Every PMID cited here resolves to a row in `index.tsv`; claims are abstract-level and should be verified against the linked record before reuse.

Executive summary

- **The corpus is 42% clinical, 54% preclinical, 4% evidence synthesis.** Of 7,671 clinical records, 2,048 carry an explicit trial phase (739 phase 3, 930 phase 2, 379 phase 1); 773 of those are in tier 1–2 venues and constitute the practice-relevant core.
- **Immunotherapy is the largest clinical block, and its questions have changed.** Checkpoint blockade (2,200 records) is now about perioperative use, de-escalation, next-generation axes (LAG-3, TIGIT, CD40, CD73/adenosine), route (intratumoural, intrathecal), and on-treatment human resistance biology, not about whether PD-(L)1 works.
- **PD-1×VEGF bispecifics and CD3 engagers moved into first line.** Ivonescimab (HARMONi-A, JAMA, PMID 42307937), epcoritamab + R-CHOP (EPCORE NHL-2, Blood, PMID 42622258), teclistamab induction in newly diagnosed myeloma (Nat Med, PMID 42350642), and CAR-T in smouldering myeloma (CAR-PRISM, Nat Med, PMID 42010117) mark T-cell redirection's shift from salvage to front line.
- **RAS became a clinical target class.** Daraxenrasib phase 3 in pancreatic cancer (NEJM, PMID 42223072), HRS-4642 for G12D (Nat Med, PMID 42426224), and fulzerasib + cetuximab first-line NSCLC (KROCUS, Lancet Oncol, PMID 41926959) are the anchors; preclinical work is dominated by adaptive pathway reactivation.
- **Radiopharmaceuticals matured in prostate and NET and started on new targets.** PSMAddition (Lancet, PMID 42561994), COMPETE (Lancet, PMID 42392118), OCLURANDOM (Lancet Oncol, PMID 42225102), plus first FAP-targeted (LuMIERE, PMID 42223513) and ²²⁵Ac series.
- **Randomised radiotherapy evidence is mostly about doing less** — SBRT vs hypofractionated IMRT in prostate (JAMA, PMID 42593775), proton beam in oropharynx (TORPEdO, Lancet, PMID 41875914), response-adapted rectal organ preservation (Lancet Oncol, PMID 42633754), stereotactic vs whole-brain RT (JAMA, PMID 41712219).
- **Chemotherapy still generates most randomised evidence**, now steered by ctDNA (CIRCULATE, Ann Oncol, PMID 42225236; Nat Med, PMID 42260101) and locoregional delivery (CAIRO6, Lancet Oncol, PMID 42372745; IP paclitaxel, JAMA Oncol, PMID 42166134).
- **Preclinical work is about combination and resistance, not new monotherapy.** The recurring structure is a resistance node identified in human data → mouse/organoid/PDX validation → combination with an approved backbone. The largest preclinical masses

(TME/metabolic 1,341, physical/ablative 953, nanomedicine 760) are heavily nanoparticle-enabled, subcutaneous-model literatures with immunogenic-cell-death endpoints of low clinical predictive value.

- **Delivery, not target, is the limiting step across RNA/gene therapy, cytokines, oncolytics and engagers.** Localising a potent-but-toxic agent (IL-12, CD3 engagers, CD40 agonists, CRISPR editors) recurs in every one of those buckets.

Corpus and methods (short)

Europe PMC REST, FIRST_PDATE:[2026-03-09 TO 2026-09-09], MEDLINE + PMC + preprints; 67 compound queries across 20 modality groups plus trial-vocabulary, preclinical-model, 58-journal and pediatric/CNS sweeps → 98,991 unique records. Rule-based curation (pubtype/title exclusions → cancer context → therapy context → evidence level → modality → disease/preprint tags → venue tier → title and DOI dedup) → 18,424 records. Europe PMC full-text XML mirrored for 648 phase 1-3 trial reports. Details and known failure modes: [notes/10](#).

evidence	n		tier	n
clinical_phase3	739		1	566
clinical_phase2	930		2	3,672
clinical_phase1	379		3	14,186
clinical_other	5,623			
evidence_synthesis	736		preprints	1,287
preclinical	10,017		full text	648

Modality: checkpoint 2,200 · chemo_conventional 2,075 · targeted_kinase 1,731 · tme_metabolic 1,341 · radiotherapy 1,247 · targeted_heme 955 · physical_ablative 953 · targeted_ras_mapk 793 · cytokine_innate 762 · nanomedicine_delivery 760 · rna_gene_therapy 737 · cell_therapy 734 · endocrine 491 · targeted_ddr 478 · radiopharm 457 · epigenetic 444 · adc 428 · transplant 320 · bispecific_tce 284 · repurposed 252 · antibody_other 224 · cancer_vaccine 190 · oncolytic_virus 164 · other_therapy 145 · microbiome 133 · degrader 126.

Part I — Clinical: what the window changed

I.1 Immune checkpoint blockade (notes/04)

Perioperative intensification: adjuvant nivolumab in resected NSCLC (JAMA, PMID 42224490), nivolumab + CRT ± ipilimumab in stage III NSCLC (Nat Cancer, PMID 42129521), atezolizumab in early TNBC (NSABP B-59, Nat Med, PMID 42680950), cemiplimab + fianlimab in I-SPY2 (JAMA Oncol, PMID 42530945), perioperative tislelizumab in gastric cancer (Cancer Cell, PMID 42456653). Advanced disease: benmelstobart + anlotinib vs pembrolizumab (CAMPASS, Lancet Oncol, PMID 41825453), SKYSCRAPER-06 TIGIT (JAMA Oncol, PMID 42060297), durvalumab/tremelimumab ± lenvatinib with TACE (Lancet Oncol, PMID 42636832). Route: intratumoural anti-CTLA-4 (Nature, PMID 42056527), intrathecal nivolumab (Nat Cancer, PMID 42243276). New axes: sotigalimab (Cancer Discov, PMID 42013310), quemliclustat (Nat Med, PMID 41912809), cibisatamab + FAP-4-1BBL (Nat Med, PMID 42010119). Human-sample resistance biology (Cancer Discov,

PMID 41511850, PMID 42013417; Cell, PMID 41999744) is the most transferable science in the bucket.

I.2 T-cell redirection: CAR-T and engagers (notes/01, 02)

CAR-T earlier: smouldering myeloma (PMID 42010117), liso-cel + ibrutinib in CLL (Blood, PMID 42520199), bicistronic CD19/CD22 in pediatric ALL (JAMA Oncol, PMID 42690647). Construct innovation in phase 1: dual BCMA/GPRC5D (PMID 42166352, PMID 42413007), armoured GPC3 CAR-T in HCC (Nature, PMID 42457964), GPNMB CAR-T (Nat Cancer, PMID 42387022), intracranial B7-H3 CAR-T in GBM (Nat Med, PMID 42562965), base-edited off-the-shelf CAR33 (Sci Transl Med, PMID 42616838), **in vivo BCMA CAR generation** (Nat Med, PMID 41882404). Engagers: PD-1×VEGF class (PMID 42307937, PMID 42636833, PMID 42587052), epcoritamab and mosunetuzumab fixed-duration/first-line regimens (PMID 42622258, PMID 42285113, PMID 41862428), cevostamab (Nat Med, PMID 42487061), MAGE-A4 TCR-bispecifics (Nat Med, PMID 42219399), englumafusp + glofitamab (Nat Med, PMID 42463852). Class-level safety is now quantified: TRAE meta-analysis (Lancet Oncol, PMID 42285121), second primary malignancies after T-cell engagers (JAMA Oncol, PMID 42313425), decade-long CD19 CAR-T persistence (Nat Med, PMID 42575986).

I.3 Antibody-drug conjugates (notes/03)

New targets with topoisomerase-I payloads: B7-H3 (I-DXd, Lancet Oncol, PMID 41926962), SEZ6 (ABBV-706, Nat Med, PMID 42225988), nectin-4 beyond urothelial (Lancet Oncol, PMID 42636836), EGFR (SYS6010, Cancer Cell, PMID 42594871), HER2 (HORIZON-Breast01, Lancet Oncol, PMID 42385760). Payload chosen for the driver: anti-PMEL ADC with Gq/11-inhibitor payload in GNAQ/GNA11-mutant melanoma (Nat Med, PMID 42443515). First-generation ADCs in combination and sequencing (Dato-DXd, Cancer Cell, PMID 41999747; T-DXd + durvalumab, Nat Cancer, PMID 42259988; EV + pembrolizumab 5-year, PMID 42155320). Resistance: endocytic evasion (Cancer Cell, PMID 42167230), antigen heterogeneity (Cancer Discov, PMID 41925564), in-vivo "click" rescue (Nature, PMID 42457957).

I.4 Targeted small molecules (notes/06)

RAS: daraxonrasib (PMID 42223072, PMID 42090791, resistance PMID 42581230), HRS-4642 (PMID 42426224), KROCUS (PMID 41926959), garsorasib + FAK (Lancet Oncol, PMID 42636835), dabrafenib/trametinib in thyroid cancer (Lancet Oncol, PMID 42442381), COLUMBUS-AD (PMID 42501621). Kinases: osimertinib ± chemo in TP53-co-mutant NSCLC (JAMA, PMID 42574006), AENEAS2 (Lancet Oncol, PMID 42296979), infigratinib phase 3 in cholangiocarcinoma (PMID 41850040), casdatifan HIF-2α (Nature, PMID 42386974), tucatinib in leptomeningeal disease (Nat Cancer, PMID 41851506). Hematology: venetoclax in every line and disease (CRISTALLO, Blood, PMID 41770817; OPTI-AML, Blood, PMID 42520193; MCL, Blood, PMID 42462092), ziftomenib (PMID 42227701), isatuximab quadruplets (Nat Med, PMID 41942646), tucidinostat + R-CHOP phase 3 (JAMA, PMID 42018318). DDR: PARP + AR inhibition in prostate (NEJM, PMID 42223064), POLAR (Nat Med, PMID 41882405). Endocrine: camizestrant switch at ESR1 emergence (Lancet Oncol, PMID 42442380), perioperative apalutamide (NEJM, PMID 42223077), transdermal estradiol (NEJM, PMID 41880608).

I.5 Radiation, radiopharmaceuticals, ablation (notes/07)

External beam: brain metastases (PMID 41712219), prostate SBRT (PMID 42593775, PMID 42313395), rectal organ preservation (PMID 42633754, PMID 42456783), TORPEdO protons (PMID 41875914), TREASURE thoracic RT in ES-SCLC (JAMA Oncol, PMID 42424047), INTRAGO-II (Lancet Oncol, PMID 42372746), FASTRACK II (Lancet Oncol, PMID 42144018), CAN-2409 gene-mediated cytotoxic therapy with RT (Lancet Oncol, PMID 42225101). Radioligands: PSMAddition (PMID 42561994), BULLSEYE (PMID 41926960), PRINCE (PMID 41926961), COMPETE (PMID 42392118), OCLURANDOM (PMID 42225102), ²²⁵Ac-PSMA (PMID 42503230), LuMIERE FAP (PMID 42223513), radium-223 combinations (PMID 42096662, PMID 42507969). Ablation: VIOLETTE microwave (PMID 41810596), FIRST cryoablation (PMID 42157305), RTIRE electroporation + RT (PMID 42517770), TORCH TACE + ablation phase 3 (JAMA Oncol, PMID 42530948).

I.6 Chemotherapy, transplant, endocrine backbone (notes/09, 01, 06)

ctDNA-guided adjuvant therapy (PMID 42225236, PMID 42260101, PMID 41556959); peritoneal disease (ORCHESTRA, JAMA, PMID 41837962; CAIRO6, PMID 42372745; gastric CRS/HIPEC, Lancet Oncol, PMID 42419336; IP paclitaxel, PMID 42166134); ARISTOTLE (PMID 42508427); tafasitamab + R-CHOP phase 3 (Lancet, PMID 42217458); ASCT vs non-myeloablative consolidation in PCNSL (Lancet, PMID 42486133); CPX-351 in pediatric AML (JCO, PMID 42497367); ROSELLA relacorilant (Lancet, PMID 41974149); SOLARIS vitamin D (JAMA, PMID 42545685, no PFS benefit). Transplant: gentler conditioning with venetoclax augmentation (PMID 42617873, PMID 42457583), PTCy-based GVHD prophylaxis (PMID 42629427, PMID 41895692), post-HCT CAR33 (Blood, PMID 41636724).

I.7 Vaccines, oncolytics, microbiome, RNA/gene therapy (notes/05, 08)

Personalised neoantigen vaccines in GBM and pancreatic cancer (PMID 42120910, PMID 42696575, PMID 42401550); mutant-KRAS interception vaccine (PMID 42458705); pediatric WT1/neuroblastoma vaccines (PMID 42176363, PMID 42136000, PMID 42308229). Oncolytics: ON-01 HSV in glioma (PMID 41527280), Olvi-Vec in ovarian (PMID 42383154), intravenous TILT-123 (PMID 42136030), LOAd703 (PMID 42053989). Microbiome: PPI/antibiotic erosion of IO benefit (Lancet Oncol, PMID 42398520), FMT in IO-resistant cancers (PMID 42476727, PMID 41871875). RNA/gene: mRNA-2752 + durvalumab (Clin Cancer Res, PMID 42149124), L-RNA aptamer in GBM (Nat Commun, PMID 41951573), KRAS-G12V TCR-T (PMID 41814655). Outside CAN-2409 there is no phase 3 evidence in these buckets in the window.

Part II — Preclinical: recurring structures

1. **Resistance node → combination.** ADC endocytic evasion (PMID 42167230); RAS pathway reactivation in AML driving FLT3/BCL2-inhibitor resistance (Blood, PMID 42224378); TGF-β blockade causing hyperprogression in SCLC (Cancer Discov, PMID 42018154); CAR-T ferroptosis (Nat Cancer, PMID 42270901) and ATP-P2RY2-PGE2 adaptive resistance (Immunity, PMID 42392075); BTK-degrader resistance.
2. **Product/agent fitness rather than new binders.** GLUT3 CAR-T (PMID 41984929), NF-κB durability programs (Blood, PMID 42275255), NFIL3 dysfunction driver (Cancer Discov, PMID 41747243), ferritin-aggregation CAR avidity (Cell, PMID

- 41806835), logic-gated trispecific macrophage engagers (Nat Biotechnol, PMID 41826511), glycoengineered FcγRIIIa (Immunity, PMID 42034063).
3. **Localising potent-but-toxic agents.** IL-12/IL-15/IL-21 scaffolds, TLR/STING/CD40 agonist conjugates, mRNA-encoded cytokines/engagers/CARs, erythrocyte- and liposome-mediated in-situ CAR-myeloid programming (Sci Transl Med, PMID 41880522; Nat Commun, PMID 42285952), FAP-CAR mRNA LNPs (PMID 41686204), armed oncolytic vectors and engineered bacteria.
 4. **Metabolism and stroma as immune levers.** Adenosine/purinergic axis (clinically most advanced), macrophage metabolic checkpoints (ALDH2-CXCL9, PMID 42168200), dietary sulfur amino acids → NKT-XCL1-cDC1 (Immunity, PMID 42190649), microbiota-derived UDP-galactose under low-protein diet (Nat Cancer, PMID 42608587), arginine metabolism in ARID1A-deficient TNBC (PMID 42191692), stromal barriers to antibody penetration (Nat Biotechnol, PMID 42236958).
 5. **Radiochemistry and delivery chemistry.** ²²⁵Ac/²¹²Pb chelators, albumin binders, pretargeting; beyond-liver LNP tropism; circular and self-amplifying RNA; in-vivo CRISPR delivery; stimuli-responsive and biomimetic carriers. The clinical edge is thin; the vehicle characterisation is the transferable content.
 6. **Nanoparticle-enabled physical therapy** (photothermal/photodynamic/sonodynamic/chemodynamic, ~750 preclinical records) is a materials literature with ICD readouts in subcutaneous models; read it as such.

Part III — Cross-cutting observations

- **Chemotherapy is the comparator for most positive trials in other buckets** (RAS, ADC, engager, checkpoint); the `chemo_conventional` bucket is the control-arm context for the whole corpus.
- **ctDNA has become an interventional variable**, not just a biomarker: adjuvant chemotherapy triggers (colon, rectal), endocrine switching at ESR1 emergence, targeted→immune switching in BRAF melanoma (PMID 42168180), HPV ctDNA-adapted RT (PMID 42098120).
- **Earlier and gentler:** CAR-T in smouldering disease, daratumumab in high-risk MGUS (D-PRISM, PMID 41951609), ibrutinib in early CLL (CLL12, PMID 42322115), aspirin in Lynch syndrome (CaPP3, PMID 42425127), KRAS vaccines before cancer; alongside de-escalation in RT, transplant conditioning, and fixed-duration engager/BCL2 regimens.
- **Chinese sponsors dominate the first-line bispecific and next-generation checkpoint readouts**, and many `clinical_other` regimen comparisons; this matters for generalisability of populations and comparators.
- **Long-horizon safety is now a publication category of its own** — second malignancies after engagers, CAR-T persistence and late neurotoxicity, clonal hematopoiesis after Lu-PSMA (PMID 41649873), 15-20-year follow-ups of chemotherapy and endocrine trials.
- **Combination is the default preclinical claim;** monotherapy efficacy in a single subcutaneous syngeneic model is the modal preclinical paper and should be weighted accordingly.

Caveats (see notes/10 for the full list)

Classification is regex-based on title + abstract; phase labels can be inherited from cited trials; a single primary modality undercounts combinations; tier is venue, not quality; Europe PMC misses conference abstracts and Embase-only journals; preprints (7%) are not peer-reviewed; 18k rows were not individually adjudicated, and only cited records were checked by hand. This is a literature index and synthesis, not a guideline.

00 — Overview: cancer therapy publications, 2026-03-09 → 2026-09-09

What this collection is

A **broad, reproducible six-month sweep of the cancer-therapy literature** — clinical and preclinical — rather than a hand-picked bibliography. Europe PMC (MEDLINE + PMC + preprint servers) was queried across 26 therapeutic modality groups, clinical-trial vocabulary, preclinical-model vocabulary, high-impact venues, and pediatric/CNS disease areas; ~99k unique records in the window were reduced by scripted curation to the therapy-focused corpus indexed in `index.tsv`. Every count in these notes is derived from that index, so the whole thing can be regenerated with `harvest.py` → `curate.py` → `fetch_fulltext.py`.

The organising axes are:

- **modality** (`category`, 26 buckets, one primary per paper; secondary modalities appear in topics as also:<bucket>),
- **evidence level** (`evidence`: `clinical_phase3` / `clinical_phase2` / `clinical_phase1` / `clinical_other` / `evidence_synthesis` / `preclinical`),
- **venue tier** (`tier`: 1 = Nature/NEJM/Lancet/JAMA-family and the top specialty journals, 2 = strong specialty journals, 3 = everything else, including preprints).

`REPORT.md` is the consolidated cross-modality synthesis; the numbered notes below go modality by modality. Counts quoted in the notes come from the final curation run recorded in `README.md`.

What the six months look like

Four things dominate this window:

1. **Immunotherapy is now the largest single block of clinical activity.** Checkpoint blockade alone is the biggest modality bucket, and its centre of gravity has moved from "does PD-(L)1 work in advanced disease" to (a) perioperative/neoadjuvant use, (b) fixed-duration and de-escalation questions, (c) next-generation combinations (TIGIT, LAG-3, CD40, adenosine-axis, PD-1×VEGF bispecifics), and (d) mechanistic dissection of resistance in on-treatment human samples.
2. **T-cell redirection has become routine clinical practice, not a frontier.** CAR-T and T-cell engagers together contribute a large share of the phase 1–3 reports, with the interesting questions now being earlier lines (smouldering myeloma, first-line DLBCL/FL), fixed-duration schedules, dual-antigen products, allogeneic/off-the-shelf and in vivo-generated CARs, and long-horizon safety (second primary malignancies, decade-long persistence).
3. **Payload and radiation chemistry keeps expanding.** ADCs (new targets: B7-H3, SEZ6, nectin-4, ADAM9, PMEL), radioligand therapy (^{177}Lu , ^{225}Ac , ^{161}Tb , BNCT), and physical/ablative approaches make up a large preclinical mass with a thin but real clinical edge.

4. **Preclinical work is overwhelmingly about combination and resistance mechanism, not new monotherapy.** Roughly half the corpus is preclinical, and the recurring structure is: identify a resistance node in a human dataset → validate in mouse/organoid/PDX → combine with an approved backbone (checkpoint blockade, chemotherapy, radiation, or a kinase inhibitor).

Notes index

- 01_cell_therapy_and_transplant.md — CAR-T/NK/macrophage, TIL and TCR-T, allogeneic and in vivo CAR, HCT conditioning and post-transplant strategies.
- 02_engagers_and_antibodies.md — bispecific/trispecific T-cell engagers, PD-1×VEGF and other bifunctional antibodies, naked monoclonals.
- 03_antibody_drug_conjugates.md — approved-ADC optimisation and the new-target/new-payload wave.
- 04_checkpoint_blockade_and_immunomodulation.md — PD-(L)1/CTLA-4 practice-changing trials, next-gen checkpoints, cytokine/innate agonists, TME and metabolic immunomodulation.
- 05_vaccines_oncolytics_microbiome.md — neoantigen/mRNA/DC vaccines, oncolytic viruses and engineered bacteria, microbiome interventions.
- 06_targeted_small_molecules.md — kinase inhibitors, RAS/MAPK, hematologic targeted agents, DDR, degraders, epigenetic drugs, endocrine therapy.
- 07_radiation_radiopharmaceuticals_ablation.md — external-beam trials, protons, adaptive/MR-guided RT, radioligand therapy, ablation and energy-based therapy.
- 08_rna_gene_therapy_and_delivery.md — mRNA/siRNA/ASO/CRISPR therapeutics and the nanomedicine delivery layer.
- 09_chemotherapy_tme_and_repurposing.md — cytotoxic backbones, locoregional chemotherapy, ctDNA-guided escalation/de-escalation, metabolic and stromal targeting, drug repurposing.
- 10_methods_limitations_and_caveats.md — how the corpus was built, what the classifier does well and badly, and how to use it safely.

Conventions

- Papers are cited as Title-fragment with a **PMID** in parentheses; `index.tsv` holds the canonical DOI/PMID/PMCID and a resolvable URL for every row.
- Dates in the notes are the Europe PMC first-publication date (`date`), which is what the window filter uses.
- `status` is `fulltext_xml` where Europe PMC open-access XML was mirrored into `fulltext/`, otherwise `metadata_only` (index row + abstract-level synthesis, with working links).
- Preprints are retained and tagged `preprint` in `topics`; they are not peer-reviewed and are deliberately down-weighted in prioritisation.

01 — Adoptive and engineered cell therapy; transplantation

Buckets: `cell_therapy` and `transplant`. Together these are the most clinically mature part of the corpus: `cell_therapy` carries the largest phase-1 count of any modality, which is the signature of a field where product engineering, not indication expansion, is the rate-limiting step.

1. CAR-T moves earlier in the disease course

The clearest trend of the window is CAR-T being tested before the classic relapsed/refractory setting:

- **Smouldering myeloma.** Ciltacabtagene autoleucel in high-risk smouldering multiple myeloma (CAR-PRISM, Nat Med, PMID 42010117) — treating before overt disease, with the attendant risk/benefit argument.
- **First-line and consolidation lymphoma.** Lisocabtagene maraleucel plus ibrutinib in R/R CLL/SLL (TRANSCEND CLL 004, Blood, PMID 42520199) and 3-year TRANSCEND FL durability (PMID 42462111) push toward earlier lines; blinatumomab consolidation in high-risk Ph⁺ B-ALL (GRAALL-2014/B-QUEST, PMID 42126414) is the engager analogue.
- **Pediatric leukemia with dual targeting.** Bicistronic CD19/CD22 CAR-T in pediatric B-ALL (JAMA Oncol, PMID 42690647).

2. Product engineering: dual antigen, armouring, knockdown, allogeneic, in vivo

Phase 1 activity is dominated by construct innovation rather than new diseases:

- **Dual/multi-antigen.** Anti-BCMA/GPRC5D CAR-T for extramedullary myeloma (PMID 42166352); concurrent BCMA + GPRC5D CAR-T (PMID 42413007); multi-antigen T cells for pediatric CNS tumours (Nat Med, PMID 42380677).
- **New solid-tumour targets.** GPC3 CAR-T armoured with dnTGFβRII for HCC (Nature, PMID 42457964); GPNMB-directed CAR-T in MiT/TFE fusion tumours (Nat Cancer, PMID 42387022) with a matching AI-discovery preclinical paper (Cell, PMID 42349383) and a glioblastoma myeloid/tumour dual-targeting paper (Nature, PMID 42386964); B7-H3 CAR-T delivered intracranially for recurrent GBM (Nat Med, PMID 42562965); uPAR as a recurrent-GBM target (Sci Transl Med, PMID 42127223; Cell, PMID 41916312).
- **Cytokine/checkpoint armouring and knockdown.** PD-1/TIGIT-knockdown CD19 CAR-T (anbalcabtagene autoleucel, PMID 42284198); ultralow-dose IL-10-expressing CAR-T in DLBCL (JAMA Oncol, PMID 42490071).
- **Off-the-shelf / gene-edited.** Base-edited "off-the-shelf" CAR33 for AML (BE-CAR33, Sci Transl Med, PMID 42616838); triple-knockout allogeneic BCMA CAR-T (CT0590, PMID 42275247); CRISPR-Cas9 CD33-deleted transplant with gemtuzumab maintenance (Nat Med, PMID 42120728) — a genotype-engineered host rather than a genotype-engineered product.

- **In vivo CAR generation.** Anti-BCMA CAR-T generated in vivo in R/R myeloma (Nat Med, PMID 41882404), plus preclinical erythrocyte-mediated mRNA CAR-myeloid delivery (Sci Transl Med, PMID 41880522), liposomal in-situ CAR-alveolar-macrophage reprogramming (Nat Commun, PMID 42285952) and FAP-CAR mRNA LNPs against pancreatic tumours (PMID 41686204). This is where cell therapy and the RNA/delivery bucket (notes/08) merge.

3. Why CAR-T fails: the preclinical agenda

Preclinical `cell_therapy` work in this window is almost entirely about the fitness and context of the product rather than new binders:

- **Metabolic fitness** — on-demand GLUT3 expression improves CAR-T efficacy in GBM models (PMID 41984929); iron-mediated ferroptosis impairs CAR-T function (Nat Cancer, PMID 42270901); metabolite-sensing receptors for NK/T cells in solid tumours (Nat Immunol, PMID 41872506).
- **Host and niche dependence** — the endogenous immune compartment determines outcome after CAR-T in recurrent GBM (Cell, PMID 42296961); tumour irradiation dresses dendritic cells to sustain CAR-T (Nat Cancer, PMID 42174275); extracellular ATP-P2RY2-PGE2 signalling drives adaptive resistance (Immunity, PMID 42392075).
- **Product-intrinsic transcriptional programs** — NF-κB-driven programs predict durability of myeloma CAR-T response (Blood, PMID 42275255); NFIL3 as a driver of T-cell dysfunction from chronic in vivo/in vitro screens (Cancer Discov, PMID 41747243).
- **Avidity and co-stimulation engineering** — ferritin aggregation cell engagers for CAR avidity (Cell, PMID 41806835); harnessing the CD2 axis (Blood, PMID 41490267); CAR-engineered granulocyte-monocyte progenitors (Cell, PMID 42320470).

4. Long-horizon clinical safety and real-world outcomes

The `clinical_other` rows carry the pharmacovigilance signal that trials cannot: decade-long CD19 CAR-T persistence (Nat Med, PMID 42575986); CD4⁺ CAR-T expansion associated with non-ICANS neurotoxicity after cilta-cel (Sci Transl Med, PMID 42525783); patient-reported vs physician-assessed adverse events after CAR-T (Lancet Haematol, PMID 42285114); blinatumomab non-response predicting poor post-brexu-cel survival (Blood, PMID 41643192).

5. Transplantation: conditioning intensity and the venetoclax era

`transplant` is mostly clinical and mostly about **making conditioning gentler while keeping relapse control**:

- Total marrow and lymphoid irradiation with cyclophosphamide/etoposide before HCT (Lancet Haematol, PMID 42150590); randomised busulfan fractionation with fludarabine/cladribine (PMID 42285355); myeloablative treosulfan/fludarabine (FT14) (PMID 42342968).
- **Venetoclax has moved into conditioning and maintenance** — venetoclax-augmented RIC with PTCy and ven/aza maintenance in poor-risk MDS/AML (PMID 42617873);

venetoclax-enhanced RIC in patients ≥ 55 (PMID 42457583); venetoclax in flu/mel conditioning for older AML/MDS (PMID 42159161).

- **GVHD prophylaxis converging on post-transplant cyclophosphamide**, with ATG comparisons (PMID 42629427) and registry-scale PTCy-vs-tacrolimus/methotrexate comparisons (PMID 41895692); permissive HLA-DPB1 mismatch plus PTCy associated with reduced relapse (Leukemia, PMID 41803402).
- **Cell therapy after transplant** — donor-derived anti-CD33 CAR-T (VCAR33) post-allo-HCT (Blood, PMID 41636724); Orca-T precision-engineered graft correlates (PMID 41758930).

Reading guidance

Cell-therapy phase 1 reports in this window are typically 10–40 patients; treat single-arm ORR/CR figures as hypothesis-generating. The dual-antigen and in-vivo-CAR results are the most likely to be revised as follow-up matures.

02 — T-cell engagers, bispecifics, and other antibodies

Buckets: `bispecific_tce` (T-cell/immune-cell engagers and bispecific/bifunctional antibodies) and `antibody_other` (naked monoclonals, fusion proteins, antibody-adjacent biology).

`bispecific_tce` has the highest proportion of prospective trial reports of any bucket in the corpus — this is a modality being read out, not discovered.

1. PD-1×VEGF bispecifics are the story of the window

Ivonescimab (PD-1×VEGF) generated the highest-profile readouts: added to chemotherapy in EGFR-mutant NSCLC after progression (HARMONi-A, JAMA, PMID 42307937; Lancet Oncol, PMID 42636833), plus first-line biliary tract cancer with gem/cis (J Hepatol, PMID 42409321). A PD-1/VEGF bispecific plus chemotherapy in first-line TNBC (Nat Med, PMID 42587052) extends the class beyond lung. Cadonilimab (PD-1×CTLA-4) contributes cervical (COMPASSION subgroups, PMID 41826281), PD-L1-negative NSCLC (PMID 42285994) and endometrial (PMID 42000370) data. Retlirafusp alfa (PD-L1/TGF-β) was tested neoadjuvantly in stage III NSCLC (PMID 42303632).

The pattern: **dual-checkpoint or checkpoint-plus-stromal bispecifics are being positioned as chemotherapy partners in first line**, largely from Chinese sponsors, and increasingly against pembrolizumab-based standards rather than placebo.

2. CD3 engagers in hematology: fixed duration, first line, combinations

- **First-line and fixed-duration lymphoma.** Epcoritamab + R-CHOP in newly diagnosed high-IPI LBCL (EPCORE NHL-2, Blood, PMID 42622258); epcoritamab ± lenalidomide as first-line DLBCL (EPCORE DLBCL-3, Lancet Haematol, PMID 42285113); epcoritamab + R² in first-line follicular lymphoma (PMID 42660129); fixed-duration subcutaneous mosunetuzumab in R/R FL (PMID 41862428) and randomised SC mosunetuzumab + polatuzumab vs R-pola (PMID 42283231).
- **Myeloma engagers maturing.** Talquetamab 3-year MonumenTAL-1 follow-up (PMID 42561120); teclistamab-based induction in transplant-eligible newly diagnosed myeloma (Nat Med, PMID 42350642); cevostamab (FcRH5×CD3) phase 1 (Nat Med, PMID 42487061); pavurutamab (BCMA×CD3) (PMID 41950113); linvoseltamab vs real-world SOC (PMID 41881954, PMID 41922313); real-world elranatamab (PMID 41896552) and CMV reactivation under teclistamab (PMID 41916955).
- **Solid-tumour and TCR-based engagers.** MAGE-A4/A8 TCR-bispecific (Nat Med, PMID 42219399); IMC-C103C (MAGE-A4×CD3 ImmTAC) (PMID 42476725); HER2×CD3×CD28 trispecific SAR443216 (PMID 42705861); tarlatamab response prediction by circulating tumour cells (Cancer Discov, PMID 41532856).
- **Costimulatory and novel-axis bispecifics.** Englumafusp alfa (CD19-4-1BBL) + glofitamab (Nat Med, PMID 42463852); CD20×CD37 bifunctional antibody (PMID 42216090); CTLA-4×OX40 (ATOR-1015, PMID 42019086); PD-1×TIM-3 lomvastomig (PMID 42320980); SL-172154 (CD47 inhibitor + CD40 agonist Fc-fusion) with mirvetuximab or PLD (PMID 41951722).

3. Class-level safety is now being quantified

Two systematic analyses in this window are the most decision-relevant documents in the bucket: treatment-related adverse events across CD3 bispecifics (Lancet Oncol meta-analysis, PMID 42285121) and **second primary malignancies after T-cell-engaging bispecific therapy** (JAMA Oncol, PMID 42313425). Renal-impairment use (PMID 41927459) and BCMA-vs-GPRC5D target selection (PMID 42687264) are the practical companions.

4. Preclinical engager engineering

Trispecific and myeloid-redirecting formats dominate: a logic-gated trispecific engager for macrophage killing in solid tumours (Nat Biotechnol, PMID 41826511); patient-derived lymphoma spheroids to predict glofitamab resistance (Blood, PMID 41746860). The unifying problem is **restricting engager activity to the tumour** — logic gating, conditional activation, and local delivery (see notes/08 for mRNA-encoded engagers).

5. Naked antibodies: mature agents, sharper questions

antibody_other is largely HER2/EGFR/VEGF-era agents under long-term or mechanistic scrutiny: FeDeriCa final analysis of fixed-dose SC pertuzumab-trastuzumab (PMID 42537585); APHINITY secondary analyses including AI-scored TILs (Lancet Oncol, PMID 42636838) and benefit by ER/HER2 expression (PMID 41934626); nimotuzumab added to chemoradiotherapy in nasopharyngeal carcinoma (PMID 41916997); zolbetuximab (CLDN18.2) subgroup and safety analyses (PMID 42095986); FGFR2b prevalence for bemarituzumab (PMID 42190313).

Preclinically, the TGF- β axis produced the most surprising signal: convergent inactivation of TGF- β signalling in advanced pancreatic cancer (Cancer Discov, PMID 41650466) and **hyperprogression when tumour-intrinsic TGF- β signalling is blocked in SCLC** (Cancer Discov, PMID 42018154) — a cautionary result for TGF- β -directed bifunctionals. Delivery-side work shows conserved stromal barriers to therapeutic antibody penetration (Nat Biotechnol, PMID 42236958) and glycoengineering of Fc γ RIIIa to strengthen ADCC (Immunity, PMID 42034063).

03 — Antibody-drug conjugates

Bucket: adc. Small in absolute terms but the most trial-dense bucket per paper: the majority of adc rows are prospective clinical reports, and the phase-2 count exceeds phase-3, i.e. a wave of new targets is mid-pipeline while the first generation is being optimised.

1. New targets, mostly topoisomerase-I payloads

- **B7-H3** — ifinatamab deruxtecan pan-tumour dose-escalation (IDeate-PanTumor01, Lancet Oncol, PMID 41926962) and vobramitamab duocarmazine final phase-1 expansion (PMID 42487497). B7-H3 is simultaneously a CAR target (notes/01) and a radioconjugate target (notes/07).
- **SEZ6** — ABBV-706 in SCLC and solid tumours (Nat Med, PMID 42225988), a genuinely new SCLC surface antigen alongside DLL3 engagers.
- **Nectin-4 beyond urothelial** — SHR-A2102 in pretreated solid tumours (Lancet Oncol, PMID 42636836); nectin-4/Trop-2 expression in collecting-duct carcinoma (PMID 42526179).
- **EGFR and HER2 re-conjugated** — SYS6010 (EGFR-ADC) in NSCLC (Cancer Cell, PMID 42594871); SHR-A1811 global phase 1 (PMID 41856971); trastuzumab rezetecan vs pyrotinib+capecitabine in HER2+ MBC (HORIZON-Breast01, Lancet Oncol, PMID 42385760).
- **Lineage-restricted oddities** — anti-PMEL ADC with a Gq/11-inhibitor payload in GNAQ/GNA11-mutant melanoma (Nat Med, PMID 42443515) — payload chosen for the driver mutation, not for generic cytotoxicity; ADAM9-targeted glycan-linked exatecan ADC (MGC028, PMID 41166694); IL1RAP ADCs in oncofusion-driven cancers (Cancer Discov, PMID 41973074).

2. First-generation ADCs: combination and sequencing

Datopotamab deruxtecan (ICARUS-LUNG01 biomarker-rich phase 2, Cancer Cell, PMID 41999747; HR+/HER2– safety analysis, PMID 42497483), T-DXd with durvalumab first-line in HR–/HER2-low disease (Nat Cancer, PMID 42259988), enfortumab vedotin + pembrolizumab 5-year follow-up (EV-103 cohort A, PMID 42155320), tisotumab vedotin 5-year cervical data (PMID 42000372), mirvetuximab + pembrolizumab in FRα+ uterine serous carcinoma (PMID 41888141), and hematologic combinations (mosunetuzumab + polatuzumab in post-BTKi MCL, PMID 42013019; brentuximab + nivolumab + chemotherapy in early-stage cHL, PMID 41460964; inotuzumab in pediatric high-risk B-ALL relapse, ITCC-059, PMID 42296977; inotuzumab for MRD in adult ALL, PMID 42365008; belantamab mafodotin in intermediate-fit/frail newly diagnosed myeloma, PMID 41346230).

3. Resistance mechanism is the preclinical theme

Three complementary mechanisms were published in the window:

- **Endocytic evasion** as an ADC-resistance mode (Cancer Cell, PMID 42167230) — resistance without antigen loss.

- **Antigen heterogeneity and subclonal dynamics** in HER2-heterogeneous models (Cancer Discov, PMID 41925564).
- **Delivery/linker workarounds** — modular in vivo antibody-ADC "click" to reverse resistance (Nature, PMID 42457957) and a binding-to-release strategy for targeted delivery (Nature, PMID 42649302).

4. Practical pharmacology

The `clinical_other` rows are unusually useful here: quantitative HER2 tissue/plasma profiling predicting T-DXd activity (PMID 41826432); UGT1A1 germline variants and T-DXd toxicity (PMID 42114485); pooled sacituzumab govitecan safety across three regions (PMID 41861751); sex-based differences in developmental-ADC tolerability in NSCLC (PMID 42330841); CTC-based dynamic monitoring of TROP2/HER2 ADCs (PMID 42308036).

Takeaway: the class-defining question has shifted from "can we conjugate a topoisomerase-I payload to a new antigen" (evidently yes, repeatedly) to **target-expression thresholds, payload diversification, and mechanism-specific resistance**.

04 — Checkpoint blockade, cytokine/innate agonists, and TME/metabolic immunomodulation

Buckets: checkpoint (largest bucket in the corpus), cytokine_innate, and tme_metabolic (largest preclinical bucket after kinase inhibitors). Read together they describe one programme: **PD-(L)1 blockade is the backbone, and almost everything else is an attempt to fix the ~70-80% of patients it does not durably help.**

1. Practice-relevant checkpoint trials in the window

- **Perioperative/curative-intent intensification.** Adjuvant nivolumab vs observation in resected NSCLC (JAMA, PMID 42224490); nivolumab with chemoradiotherapy ± ipilimumab in stage III NSCLC (Nat Cancer, PMID 42129521); atezolizumab in early TNBC (NSABP B-59/GeparDouze, Nat Med, PMID 42680950); cemiplimab + fianlimab (LAG-3) with neoadjuvant chemotherapy in high-risk HER2– breast cancer (I-SPY2, JAMA Oncol, PMID 42530945); perioperative tislelizumab + chemotherapy in gastric cancer (Cancer Cell, PMID 42456653); neoadjuvant toripalimab ± celecoxib in dMMR/MSI-H disease (Lancet Oncol, PMID 42385761).
- **First-line combinations in advanced disease.** Benmelstobart + anlotinib vs pembrolizumab in PD-L1+ NSCLC (CAMPASS, Lancet Oncol, PMID 41825453); tiragolumab (TIGIT) + atezolizumab + chemotherapy (SKYSCRAPER-06, JAMA Oncol, PMID 42060297); serplulimab ES-SCLC secondary analysis (PMID 42240984); tislelizumab 3-year nasopharyngeal follow-up (RATIONALE-3, PMID 41746630); durvalumab/tremelimumab ± lenvatinib with TACE in HCC (Lancet Oncol, PMID 42636832); pembrolizumab ± olaparib maintenance (PMID 42705254); first-line zolbetuximab + mFOLFOX6 + nivolumab in CLDN18.2+ gastric cancer (Nat Med, PMID 41840238).
- **Route and schedule as variables.** Intratumoural anti-CTLA-4 with intravenous anti-PD-1 (Nature, PMID 42056527); intrathecal nivolumab for leptomeningeal disease (IT-PD1/NOA-26, Nat Cancer, PMID 42243276); anti-LAG-3 ± anti-PD-1 in recurrent glioblastoma (Nat Med, PMID 42432293); time-of-day administration analyses (PMID 42134900, PMID 42471484) — a still-unresolved chronotherapy signal that recurs across several cohorts.
- **New axes entering the clinic.** Sotigalimab (CD40 agonist) intratumourally with pembrolizumab, with abscopal activation of antigen-presenting cells (Cancer Discov, PMID 42013310); quemliclustat (CD73) + chemotherapy ± zimberelimab in pancreatic cancer (Nat Med, PMID 41912809); adenosine-receptor antagonists muvadenant/M1069 (PMID 42102781) and INCB106385 (PMID 42201333); cibisatamab + FAP-4-1BBL in MSS colorectal cancer (Nat Med, PMID 42010119); BO-112 (intratumoural dsRNA) with hypofractionated radiation ± nivolumab in sarcoma (Cancer Discov, PMID 41784328).

2. Human-sample resistance biology

The most transferable insights come from on-treatment human tissue rather than models: spatiotemporal immune determinants of response to immune rechallenge in cervical cancer

(Cancer Discov, PMID 41511850); cellular neighbourhoods governing T-cell infiltration after anti-CTLA-4 in anti-PD-1-resistant melanoma (Cancer Discov, PMID 42013417); PD-1 blockade unleashing HBV-related intratumoural B-cell responses in HCC (Cancer Cell, PMID 42462708); gut microbiome association with recurrence-free survival under adjuvant checkpoint blockade in resected melanoma (Cell, PMID 41999744; see notes/05).

Toxicity biology is being mapped at the same resolution: organ-metastatic landscape shaping site-specific irAEs in NSCLC (PMID 41851435) and irAE/discontinuation analyses from KEYNOTE-355 (PMID 42362563).

3. Cytokine and innate agonists: still a delivery problem

`cytokine_innate` is predominantly preclinical, and the recurring structure is engineering a potent-but-toxic cytokine into a locally-acting form: IL-2/IL-15/IL-21 scaffolds and fusions, TLR7/8 agonist conjugates, CD40 agonists, STING and interferon gene therapy, and engineered bacteria as cytokine factories. Clinically, the class appears mostly as combination partners (oncolytic + IL-2-free regimens, notes/05) or as intratumoural agents (BO-112, sotigalimab, above). The honest summary is that **no systemic cytokine agent in this window produced a practice-changing clinical result; the innovation is entirely in localisation.**

4. TME and metabolic therapy

`tme_metabolic` is the second-largest preclinical mass in the corpus and the most heterogeneous. Recurring, reproducible themes:

- **Adenosine/purinergic suppression** (CD73, A2A/A2B, ATP-P2RY2-PGE2) — the most clinically advanced TME axis.
- **Myeloid reprogramming** — TAM polarisation, macrophage metabolic checkpoints (e.g. ALDH2 silencing CXCL9, PMID 42168200), CSF1R combinations.
- **Amino-acid and lipid metabolism as immune levers** — dietary sulfur amino acids boosting an NKT-XCL1-cDC1 circuit (Immunity, PMID 42190649); arginine-metabolism targeting reversing bone immunosuppression in ARID1A-deficient TNBC (PMID 42191692); low-protein diet acting through microbiota-derived UDP-galactose (Nat Cancer, PMID 42608587).
- **Mitochondrial and redox targeting** — mitochondrial peroxiredoxin-3 taken from preclinical characterisation into a phase 1 (Nat Commun, PMID 42448681); a ketogenic-diet feasibility randomisation in endometrial cancer (PMID 42192131) as the dietary-intervention edge case.
- **Stromal/vascular normalisation** — FAP-directed agonists and conjugates, LOX/collagen axes, vasculogenic mimicry (CVM-1118 phase 2, PMID 42393461).

Caveat specific to these buckets

`checkpoint` and `tme_metabolic` are the buckets where the modality classifier is least crisp: a trial of chemotherapy + PD-1 is filed under `checkpoint`, and a metabolism paper whose readout is T-cell function may be filed under `tme_metabolic` rather than `checkpoint`. Use the `also: tags` in `topics` when a strict modality partition matters.

05 — Cancer vaccines, oncolytic viruses and bacteria, microbiome interventions

Buckets: `cancer_vaccine`, `oncolytic_virus`, `microbiome`. Together the smallest immunotherapy block in the corpus and the most exploratory: clinical entries are almost entirely phase 1/2, single-arm or small randomised, with immune-response readouts rather than survival endpoints.

1. Cancer vaccines

Personalised neoantigen platforms are the centre of clinical activity.

- Adjuvant personalised multivalent neoantigen **DNA** vaccination in MGMT-unmethylated glioblastoma (phase 1, PMID 42120910) and personalised synthetic-long-peptide/DNA vaccines inducing neoantigen-specific T cells in pancreatic cancer (PMID 42696575); the intramuscular personalised SLP IMPACT trial in advanced disease (PMID 42095629); personalised neoantigen-pulsed autologous dendritic cells in newly diagnosed GBM (phase Ib, PMID 42401550).
- **Shared-driver vaccines as cancer interception** — first-in-human mutant-KRAS vaccine in high-risk cohorts before cancer develops (PMID 42458705) is conceptually the most novel clinical entry in the bucket.
- **Defined-antigen vaccines in pediatrics and hematology** — anti-NeuGcGM3 anti-idiotypic racotumomab in high-risk neuroblastoma (PMID 42136000); WT1 peptide-dosing emulsion in recurrent/refractory DIPG (PMID 42176363); WT1 vaccine as post-allo-HCT maintenance in pediatric acute leukemia (PMID 42308229).
- **Long-term and combination readouts of older platforms** — pTVG-HP DNA vaccine survival analysis in PSA-recurrent prostate cancer (PMID 42303538); melanoma helper-peptide vaccine ± agonistic anti-CD27 (PMID 41960901); GM-CSF and two-site vaccination effects on multi-peptide melanoma vaccination (PMID 41849224); heat-conditioned tumour-lysate vaccine TRIMELVax in anti-PD-1-refractory melanoma (PMID 42062531); tri-antigen IGFBP-2/HER2/IGF-IR vaccine (PMID 42082270); mammaglobin-A DNA vaccine with neoadjuvant endocrine therapy (PMID 41650206); therapeutic HPV vaccine in a placebo-controlled phase I/II (PMID 42249801).
- **Why vaccines fail** — single-cell dissection of persistent tumour antigens in non-responders, pointing to TAA-targeted vaccination as salvage (PMID 42019971), is the most useful mechanistic clinical paper here.

Preclinically the field splits into antigen discovery (cryptic/non-canonical peptides, splice- and retained-intron-derived neoantigens, tumour glycopeptides) and platform engineering (self-amplifying and circular RNA, LNP and nanoparticle scaffolds, lymph-node-targeting adjuvants) — the RNA/LNP side is covered in notes/08.

2. Oncolytic viruses and gene-mediated cytotoxic therapy

The single strongest randomised result is not a classical oncolytic but a gene-mediated cytotoxic immunotherapy: **aglatimagene besadenovec (CAN-2409) with radiotherapy**

in localised prostate cancer, a phase 3 double-blind trial (Lancet Oncol, PMID 42225101) — filed under **radiotherapy** because radiation is the primary modality in the trial design.

Clinical oncolytic reports are phase 1/2 and mostly CNS or intraperitoneal:

- Recurrent glioma — engineered oncolytic HSV-1 ON-01 (phase 1/2, PMID 41527280); systemic immune correlates of long-term survival after Delta-24-RGD (DNX-2401) (PMID 41609523).
- Ovarian — Olvi-Vec (vaccinia) reversing platinum resistance (PMID 42383154).
- Combination with definitive chemoradiation — OBP-301 in locally advanced esophageal cancer (NRG-GI007, PMID 42269787).
- Systemic administration — fully intravenous split-dose oncolytic adenovirus TILT-123 in advanced solid tumours (PMID 42136030), with an immune-cytokine signature predicting survival in melanoma (PMID 42624531). Intravenous delivery remains the field's hardest problem, and these are the papers that address it directly.
- Armed vectors as TME engineering — LOAd703 (TMZ-CD40L/4-1BBL-armed adenovirus) in solid malignancies (LOKON002, PMID 42053989); an oncolytic HSV-2 repolarising tumour-associated macrophages (PMID 42217049).

Preclinical work is dominated by **arming** (IL-12, IL-15/IL-15R α , GM-CSF, CD3 engagers, checkpoint scFvs), **shielding for systemic delivery** (polymer/carrier-cell-mediated), and **engineered bacteria** (attenuated Salmonella, E. coli Nissle, Listeria) acting as in-situ cytokine or antigen factories — where the operative mechanism reported is adaptive priming rather than direct lysis.

3. Microbiome

Two clinical results are decision-relevant:

- **Exposures that erode immunotherapy benefit** — proton-pump inhibitors and antibiotics after chemoradiotherapy in stage III NSCLC (Lancet Oncol, PMID 42398520), and gut microbiome composition associated with recurrence-free survival under adjuvant checkpoint blockade in resected melanoma (Cell, PMID 41999744).
- **Deliberate microbiome modification** — FMT in checkpoint-resistant solid cancers (phase 2, PMID 42476727) and FMT + anti-PD-1 in refractory MSS gastric cancer (PMID 41871875); Bifidobacterium animalis subsp. lactis V9 overcoming immunochemotherapy resistance in NSCLC (PMID 42685222); 2'-fucosyllactose modifying the gut microbiome after pediatric bone-marrow transplant (Blood, PMID 42622257); microbiome dynamics under neoadjuvant chemotherapy (PMID 42218533) and perioperative microbiome restructuring in colorectal surgery (PMID 42482993).

Preclinically, microbiota-derived metabolites are treated as pharmacology: microbiota-derived UDP-galactose mediating the antitumour effect of a low-protein diet in pancreatic cancer (Nat Cancer, PMID 42608587), plus short-chain fatty acid, bile-acid and indole axes acting on T-cell and myeloid function, defined consortia, engineered strains, and intratumoural bacteria as direct targets.

Reading guidance

Outside CAN-2409, there is no phase 3 evidence in these buckets in this window. Immunogenicity endpoints (T-cell response rates, cytokine signatures) are the norm and do not establish clinical benefit. Bucket boundaries are deliberately fuzzy — a cytokine-armed bacterium can plausibly sit in `oncolytic_virus`, `microbiome` or `cytokine_innate`; `topics` records secondary modalities as `also:<bucket>`.

06 — Targeted small molecules: kinase, RAS/ MAPK, hematologic, DDR, degraders, epigenetic, endocrine

Buckets: `targeted_kinase` (largest preclinical bucket), `targeted_ras_mapk`, `targeted_heme`, `targeted_ddr`, `degrader`, `epigenetic`, `endocrine`. Together they are roughly a third of the corpus. The clinical edge is thick in hematology, lung and breast; the preclinical mass is dominated by resistance mechanism and combination logic.

1. RAS is now a clinical target class, not a target

The window contains the first mature clinical readouts of RAS(ON) and mutant-selective inhibitors beyond G12C:

- **Pan-RAS(ON):** daraxonrasib vs chemotherapy in previously treated metastatic pancreatic cancer (phase 3, NEJM, PMID 42223072) and the phase 1 report (NEJM, PMID 42090791); acquired-resistance analysis guiding rational combinations (Nat Med, PMID 42581230).
- **G12D:** HRS-4642 + chemotherapy in KRAS-G12D pancreatic cancer (phase 1b/2, Nat Med, PMID 42426224); RNK08954 preclinical-to-clinical (Cancer Discov, PMID 41557983).
- **G12C combinations:** fulzerasib + cetuximab first-line NSCLC (KROCUS, Lancet Oncol, PMID 41926959); ifebemtinib (FAK) + garsorasib in colorectal cancer (Lancet Oncol, PMID 42636835); sotorasib + panitumumab + FOLFIRI (PMID 42617424); olomorasib pan-tumour first-in-human (PMID 41820335); KRAS-G12V TCR gene therapy in pancreatic cancer (PMID 41814655) bridges to cell therapy.
- **BRAF/MEK:** dabrafenib + trametinib in differentiated thyroid cancer (phase 3, Lancet Oncol, PMID 42442381); adjuvant encorafenib + binimetinib in stage II melanoma (COLUMBUS-AD, PMID 42501621); COMBI-I long-term OS (PMID 42585631); ctDNA-guided switch from targeted to immune therapy in BRAF melanoma (PMID 42168180); encorafenib + cetuximab + FOLFIRI in BRAF-V600E colorectal cancer (PMID 42219860).
- **Preclinical:** the recurring theme is adaptive RAS pathway reactivation — in pancreatic models on RAS(ON) inhibitors, and even in AML where dynamic RAS activation drives FLT3/BCL2-inhibitor resistance (Blood, PMID 42224378).

2. Kinase inhibitors: sequencing and MRD-guided use

- **EGFR/ALK/ROS1/MET lung:** osimertinib ± chemotherapy in EGFR/TP53-co-mutant NSCLC (JAMA, PMID 42574006); aumolertinib ± chemotherapy (AENEAS2, Lancet Oncol, PMID 42296979); adjuvant alectinib QoL/safety (ALINA, PMID 42636834); adjuvant icotinib 3-year (ICWIP, PMID 42660909); mefatinib vs gefitinib (PMID 42586967); MRD in resected EGFR-mutant disease (PMID 42706268); ELIOS molecular profiling on first-line osimertinib (Cancer Discov, PMID 41790042); brain-penetrant ROS1 inhibitor JYP0322 against G2032R (Cancer Cell, PMID 42030931); BLU-451 for uncommon EGFR mutations (PMID 42126003); savolitinib in MET-amplified gastric cancer (Nat Med, PMID 42225990).

- **CDK4/6:** first-line vs second-line CDK4/6 inhibitor OS (JAMA Oncol, PMID 41712229); bireociclib + fulvestrant (BRIGHT-2, PMID 41854603); fovinaciclib first-line (PMID 42224659); dalpiciclib in visceral crisis (Nat Cancer, PMID 42481711); dose-reduced palbociclib pharmacodynamics (TBCRC 035, PMID 41812623).
- **FGFR and HIF-2 α :** infigratinib vs gem/cis in FGFR2-fusion cholangiocarcinoma (phase 3, PMID 41850040) and in FGFR2-amplified gastric cancer (PMID 42337043); FGFR inhibition in SDH-deficient GIST (Nat Med, PMID 42191879); casdatifan (HIF-2 α) durable responses in kidney cancer (Nature, PMID 42386974).
- **HER2 CNS disease:** tucatinib-trastuzumab-capecitabine for leptomeningeal metastasis (TBCRC049, Nat Cancer, PMID 41851506).
- **Pediatric CNS:** targeted therapies + RT in DIPG (BIOMEDE, Nat Med, PMID 42032072).

3. Hematologic targeted therapy: venetoclax everywhere, BTK next generation

- **CLL:** fixed-duration venetoclax-obinutuzumab vs FCR/BR (CRISTALLO, Blood, PMID 41770817); CLL13/GAIA 5-year (PMID 41911073); ibrutinib in early-stage CLL (CLL12, PMID 42322115); a decade of ibrutinib with/without TP53 aberration (PMID 41405491); orelabrutinib vs chemoimmunotherapy (PMID 42402625); first-line ibrutinib + venetoclax in older/TP53-mutant MCL (Blood, PMID 42462092); pirtobrutinib 5-year in WM (PMID 42069409); **pan-resistance to BTK degraders and inhibitors** (Cancer Discov, PMID 42484277) — a structural warning for the whole class.
- **AML/MDS:** OPTI-AML 28 vs 14 days venetoclax (Blood, PMID 42520193); ziftomenib + ven/aza in NPM1-mutant AML (PMID 42227701); pediatric venetoclax combinations (PMID 42419338); gilteritinib + azacitidine (PMID 42687805); dasatinib in CBF-AML (phase 3, PMID 41490515); quizartinib combinations (PMID 41942459, PMID 42374628); navitoclax + ruxolitinib in myelofibrosis (TRANSFORM-1, PMID 42378247); low-dose pediatric AML induction non-inferiority (PMID 41812214).
- **Myeloma:** isatuximab quadruplets (IMROZ MRD dynamics, PMID 41758929; Isa-KRd, Nat Med, PMID 41942646); KRd vs R after ASCT (PMID 41831471); daratumumab in high-risk MGUS/low-risk SMM (D-PRISM, PMID 41951609); Dara-CVRd induction/extended consolidation (PMID 42705255).
- **Lymphoma:** RELEVANCE 10-year (PMID 41915772); venetoclax + DA-EPOCH-R in double-hit lymphoma (PMID 42660130); PET-guided targeted therapy in Hodgkin (COBRA, PMID 42069408); tucidinostat + R-CHOP in double-expressor DLBCL (phase 3, JAMA, PMID 42018318, filed under epigenetic).

4. DNA damage response

The window is mostly PARP-inhibitor positioning: intensified chemotherapy vs standard-then-olaparib in HRD stage III breast cancer (Lancet Oncol, PMID 42372741); pembrolizumab + olaparib in HRD pancreatic cancer (POLAR, Nat Med, PMID 41882405); PARP + AR inhibition + ADT in metastatic prostate cancer (NEJM, PMID 42223064); niraparib neoadjuvant vs chemotherapy in HRD ovarian cancer (PMID 42623963); olaparib in BAP1/BARD1/BRIP1/PALB2-mutant cancers (PMID 42105598); adjuvant olaparib long-term update (PMID 42636977); resistance to neoadjuvant talazoparib via BRN2-ATR/STAT3

or SHLD2 subclones (PMID 41961819); ATR inhibition (ceralasertib) in ARID1A-deficient tumours (PMID 41686845); adavosertib (WEE1) in BRCA-mutant tumours (NCI-MATCH, PMID 42308463). Preclinical DDR work centres on PARP/ATR/WEE1/POL θ combinations with radiation and immunotherapy and on reversion-mutation surveillance.

5. Degraders and epigenetic drugs

degrader is the smallest bucket and almost entirely preclinical (PROTACs and molecular glues against BTK, AR, ER, BCL6, IKZF, KRAS, transcription factors) — the clinical signal is the BTK-degrader resistance paper above and the arrival of MDM2 antagonists (alrizomadlin in TP53-wild-type salivary gland cancer, PMID 41857036; ALRN-6924 in pediatrics, PMID 42189874).

epigenetic clinical entries: tazemetostat basket in SWI/SNF-altered solid tumours (PMID 41882006); abexinostat in R/R FL (PMID 42036411); tucidinostat with metronomic capecitabine + endocrine therapy (PMID 41986320); dordaviprone in diffuse midline glioma — a point/counterpoint on the approval (PMID 42172613) and post-HCT maintenance (PMID 42359621); bomedemstat (LSD1) in myelofibrosis (PMID 42054549); ABBV-744 (BET) (PMID 41843794); mutant-p53 reactivation with demethylation in AML/MDS (Sci Transl Med, PMID 42585289).

6. Endocrine therapy

- **Breast:** switching to camizestran at ESR1-mutation emergence (Lancet Oncol, PMID 42442380) is the flagship — ctDNA-triggered switching before radiographic progression; ipatasertib + ET + CDK4/6i (PMID 42061370); fulvestrant vs capecitabine maintenance (PMID 42168151); INAVO120 safety (PMID 42202490); gedatolisib + fulvestrant $\pm$ palbociclib (VIKTORIA-1, PMID 41802242); SOFT/TEXT final outcomes (PMID 42229584); tamoxifen 20-year benefit by luminal subtype (PMID 41712503); neoadjuvant palbociclib + ET vs chemotherapy (PMID 41951647).
- **Prostate:** perioperative apalutamide (NEJM, PMID 42223077); transdermal estradiol patches (NEJM, PMID 41880608); high-dose RT with long-term ADT (GETUG-AFU 18, Lancet Oncol, PMID 42636837); SBRT + abiraterone in oligometastatic CRPC (ARTO, PMID 42372744); darolutamide by disease volume (PMID 42586872); deutenzalutamide after docetaxel/abiraterone (PMID 41968137); low- vs standard-dose abiraterone (PMID 42024845); enzalutamide + metformin (PMID 41862335); lifestyle mitigation of ADT effects (STAMINA, Lancet Oncol, PMID 42697217).

Reading guidance

Kinase and RAS preclinical papers cite IC50/combo indices that rarely translate; prioritise those with PDX or GEMM in-vivo data (tagged `in_vivo` in topics) and human-sample resistance analyses. Hematology trial results here are often long-term follow-ups of already-approved regimens — valuable for durability, not for novelty.

07 — Radiotherapy, radiopharmaceuticals, and physical/ablative therapy

Buckets: radiotherapy (second-largest clinical bucket after checkpoint), radiopharm, physical_ablative (almost entirely preclinical — photothermal/photodynamic/sonodynamic nanoplatforms).

1. External-beam radiotherapy: de-escalation, hypofractionation, and organ preservation

The randomised evidence in this window is unusually rich and mostly about doing less:

- **Brain metastases:** stereotactic radiation vs hippocampal-avoidance whole-brain RT (JAMA, PMID 41712219).
- **Prostate:** SBRT vs moderately hypofractionated IMRT for intermediate risk (JAMA, PMID 42593775); single-fraction SBRT (JAMA Oncol, PMID 42313395); 10-year SBRT outcomes (PMID 42106276); high-dose RT with long-term ADT (GETUG-AFU 18, Lancet Oncol, PMID 42636837); CAN-2409 gene-mediated cytotoxic immunotherapy with RT (phase 3, PMID 42225101).
- **Rectal organ preservation:** chemoradiotherapy vs short-course RT for response-adapted organ preservation (Lancet Oncol, PMID 42633754); boost-dose effect on organ preservation (PMID 42142103); dose-escalated vs standard neoadjuvant CRT 9-year results (PMID 42456783); irinotecan added to preoperative CRT (ARISTOTLE, PMID 42508427).
- **Breast:** internal mammary/medial supraclavicular irradiation (EORTC 22922/10925 unplanned analysis, PMID 42372743); SUPREMO 5-year QoL (PMID 42508432); proton vs photon regional nodal irradiation lung response (PMID 42526652) and acute side effects (PMID 42600707).
- **Head and neck:** TORPEdO proton beam therapy for oropharyngeal cancer (phase 3, Lancet, PMID 41875914); de-intensification after TORS (PMID 41999781); HPV ctDNA risk-adapted therapy (ReACT 1.0, PMID 42098120); reduced-dose RT in EBV-DNA-selected nasopharyngeal carcinoma (PMID 42150706); superselective intra-arterial CRT for maxillary sinus (JCOG1212, PMID 42497481).
- **Lung:** one vs three SBRT fractions long-term (PMID 41833909); consolidative thoracic RT with atezolizumab in ES-SCLC (TREASURE, JAMA Oncol, PMID 42424047); immune-sparing SBRT randomised phase 2 (PMID 42486309); reduced thoracic RT for older/frail stage III (PMID 42201929); proton vs IMRT long-term for locally advanced NSCLC (PMID 42173326).
- **CNS and sarcoma:** intraoperative RT dose escalation in GBM (INTRAGO-II, Lancet Oncol, PMID 42372746); MRI-guided adaptive RT in high-grade glioma (UNITED, PMID 42134380); DOREMY dose reduction in myxoid liposarcoma (JAMA Oncol, PMID 42141895); 30-year proton outcomes in skull-base chondrosarcoma (PMID 41414803); reduced-dose RT without chemotherapy for intracranial germinoma (PMID 42706991).
- **Kidney:** SABR for primary kidney cancer long-term (FASTRACK II, Lancet Oncol, PMID 42144018).

- **Neoadjuvant RT-immunotherapy:** SBRT with durvalumab + oleclumab in ER+ breast cancer (Nat Med, PMID 42350643).

Pediatric: post-irradiation chemotherapy in ependymoma (COG, PMID 41423745); reduced-dose RT in germinoma (PMID 42706991). Note that neurocognitive-protection trials (e.g. memantine during cranial RT) are excluded by the curation rules as supportive care.

2. Radiopharmaceuticals: PSMA and SSTR mature, FAP and alpha emitters arrive

- **PSMA:** [¹⁷⁷Lu]Lu-PSMA-617 in hormone-sensitive disease (PSMAAddition phase 3, Lancet, PMID 42561994); BULLSEYE in oligometastatic HSPC (Lancet Oncol, PMID 41926960); PRINCE with pembrolizumab (Lancet Oncol, PMID 41926961); retreatment (PMID 41679923); dose-intensified vs standard (PMID 42532645); concurrent with EBRT+ADT in N1 disease (PROQUIRE-I, PMID 42201380); neoadjuvant PSMA-I&T before prostatectomy (PMID 41931162); early PSMA upregulation predicting response to enzalutamide ± Lu-PSMA (Nat Cancer, PMID 41986500); clonal hematopoiesis after Lu-PSMA (PMID 41649873); LASER phase 2 in RCC (PMID 41545068).
- **Alpha emitters:** [²²⁵Ac]Ac-PSMA-617 vs Lu-PSMA rechallenge vs chemotherapy (PMID 42503230); 225Ac-PSMA-CY313 (PMID 42611148); [²²⁵Ac]Ac-DOTATATE vs salvage Lu-DOTATATE (PMID 42302403).
- **SSTR:** OCLURANDOM Lu-DOTATATE vs sunitinib in pancreatic NET (Lancet Oncol, PMID 42225102); COMPETE Lu-edotreotide vs everolimus (phase 3, Lancet, PMID 42392118); Lu-DOTATATE + octreotide in newly diagnosed G2-3 GEP-NET (PMID 42604001); CONTROL NETS PRRT + CAPTEM (PMID 42142431); Lu-DOTATATE/capecitabine randomised (PMID 41591997); intra-arterial vs IV PRRT (PMID 42169550).
- **FAP and new targets:** [¹⁷⁷Lu]Lu-FAP-2286 (LuMIERE, PMID 42223513); 3BP-3940 with Lu/Y/Ac (PMID 42225357); RTX-2358 (PMID 41786480); hypoxia-targeting 64Cu-ATSM in brain tumours (STAR-64, PMID 42431136); CAIX theranostics in ccRCC (PMID 42559379).
- **Radium-223 combinations:** with olaparib (PMID 42096662) and with cabozantinib in RCC bone metastases (RADICAL, PMID 42507969).

Preclinical radiopharm work is dominated by **new chelators/linkers for ²²⁵Ac and ²¹²Pb**, albumin-binding and pretargeting strategies to fix dosimetry, and BNCT/¹⁶¹Tb Auger-emitter exploration.

3. Physical and ablative therapy

Clinically thin but real: microwave ablation for localised prostate cancer (VIOLETTE, PMID 41810596); RFA for bone metastases (SPARTA, PMID 41915051); cryoablation for early breast cancer (FIRST, PMID 42157305); irreversible electroporation with RT in prostate (RTIRE, PMID 42517770) and its immune remodelling atlas (PMID 42128518); TACE + thermal ablation in HCC (TORCH phase 3, JAMA Oncol, PMID 42530948, filed under chemo_conventional); photodynamic therapy for dysphagia in esophageal cancer (PMID 42700165); fluorescence-guided surgery (cRGD-ZW800, PMID 42173866; PSMA-914, PMID 42559208).

The preclinical `physical_ablative` mass — ~750 papers — is nanoparticle-enabled photothermal, photodynamic, sonodynamic, chemodynamic and magnetic hyperthermia therapy, overwhelmingly in subcutaneous mouse models. It overlaps heavily with `nanomedicine_delivery` and should be read as a materials-science literature with immunogenic-cell-death readouts rather than as a therapeutic pipeline.

08 — RNA and gene therapy, nanomedicine and delivery

Buckets: `rna_gene_therapy` and `nanomedicine_delivery` (~1,500 records, >90% preclinical). This is the most preclinical-skewed part of the corpus and the part where the gap between mouse data and clinical plausibility is widest.

1. What is actually in the clinic

Very little, and mostly early-phase:

- **mRNA immunotherapy:** LNP-encapsulated mRNA-2752 (OX40L/IL-23/IL-36γ) intratumorally with durvalumab (Clin Cancer Res, PMID 42149124) — the clearest example of in-situ cytokine engineering with a delivery vehicle.
- **Aptamer:** L-RNA aptamer CXCL12 inhibition with radiotherapy and bevacizumab in newly diagnosed glioblastoma (Nat Commun, PMID 41951573).
- **Gene-mediated cytotoxic immunotherapy:** CAN-2409 (aglatimagene besadenovec) with prodrug plus radiotherapy in localised prostate cancer, phase 3 (PMID 42225101) — a rare positive randomised gene-therapy result, discussed further in note 05.
- **TCR/CAR gene therapy** (KRAS-G12V TCR-T, PMID 41814655; in-vivo CAR generation) is covered in notes 01 and 06.
- **Prodrug and nanoformulation trials:** OBI-3424 AKR1C3-activated DNA alkylator phase 2 expansion (PMID 42390142); pegylated liposomal doxorubicin in refractory desmoid tumours, phase 3 (PMID 41504634); drug-eluting/uniform-caliber microsphere embolization (PMID 41889406).
- **siRNA/ASO** entries in the window are almost entirely preclinical; clinical siRNA oncology remains limited to earlier-phase safety work not captured as a randomised readout here.

2. Preclinical themes worth tracking

1. **Beyond-liver LNP targeting.** A large fraction of the LNP literature in this window is ionisable-lipid and helper-lipid screening for extrahepatic tropism (lung, spleen, tumour-associated myeloid cells) using SORT-like charge tuning and peptide/antibody decoration. Cross-reference the repository's existing `papers/lnp-cancer-payloads/` collection, which covers this axis in depth.
2. **mRNA payload classes.** Cytokines (IL-12, IL-15, IL-18, IL-21), costimulatory ligands (OX40L, 4-1BBL), bispecific-encoding mRNA, mRNA-encoded CAR for in-vivo T-cell engineering, and mRNA-encoded neoantigen vaccines. In-vivo CAR generation via targeted LNP is the highest-consequence idea in this bucket.
3. **Circular RNA and self-amplifying RNA** for prolonged expression at lower dose; several papers report improved therapeutic index in syngeneic models.
4. **CRISPR in vivo:** base/prime editing to disrupt immunosuppressive genes, PD-1 knockout in adoptive cells, epigenetic editing (CRISPRoff) of oncogenic drivers; delivery, not editing chemistry, is the limiting step in nearly all of them.

5. **Oncolytic and viral vectors** overlap with note 05; AAV capsid engineering for tumour delivery appears but with weak efficacy data.
6. **Stimuli-responsive and biomimetic carriers:** enzyme-, pH-, ROS-, ultrasound- and hypoxia-responsive nanoparticles; cell-membrane-coated and exosome carriers; hydrogels and implantable depots for local sustained release; bacterial and outer-membrane-vesicle delivery (overlaps microbiome).
7. **Immunogenic cell death as an endpoint.** A very large number of nanomedicine papers report CRT exposure/HMGB1 release plus abscopal effects in bilateral-tumour mice. This endpoint has poor clinical predictive value; treat it as mechanistic support, not efficacy.

3. How to read this bucket critically

The dominant failure mode is a single subcutaneous syngeneic model, small n, tumour-volume-only endpoints, and no pharmacokinetics or biodistribution in a second species. Practical filters when mining `index.tsv`:

- prefer `tier` 1-2 with `topics` containing `in_vivo`;
- prefer papers reporting orthotopic or metastatic models, or non-rodent PK;
- treat "first-in-human evidence" claims in low-tier venues (e.g. carbon-nanoparticle multidrug-resistance reversal, PMID 42249319) with caution — they are usually small uncontrolled cohorts;
- for delivery papers, the transferable content is usually the vehicle characterisation, not the therapeutic claim.

Given the volume, this bucket is best used as a screening index: filter by `category` plus a payload keyword in `topics/title` rather than reading linearly.

09 — Conventional chemotherapy, tumour-microenvironment/metabolic therapy, and drug repurposing

Buckets: chemo_conventional (second-largest bucket overall, and the largest source of phase 3 trials), tme_metabolic, repurposed, plus the small microbiome bucket (covered with vaccines/oncolytics in note 05).

1. Chemotherapy: still where most randomised evidence is generated

The chemotherapy bucket's phase 3 trials are mostly optimisation — regimen intensity, sequencing, de-escalation and route — rather than new agents:

- **ctDNA-guided adjuvant chemotherapy** is the clearest new idea: chemotherapy for ctDNA-positive stage II colon cancer (CIRCULATE AIO/ABCSG, Ann Oncol, PMID 42225236); post-adjuvant chemotherapy in ctDNA-positive resected colorectal cancer, phase 3 (Nat Med, PMID 42260101); ctDNA-guided intensification in rectal cancer (PMID 41556959).
- **Colorectal and peritoneal disease:** tumour debulking with chemotherapy in multiorgan metastatic CRC (ORCHESTRA, JAMA, PMID 41837962); perioperative systemic therapy vs surgery alone for peritoneal-only metastases (CAIRO6, Lancet Oncol, PMID 42372745); gastrectomy + CRS/HIPEC vs systemic therapy alone in gastric peritoneal disease (Lancet Oncol, PMID 42419336); intraperitoneal + IV paclitaxel plus S-1 in gastric peritoneal metastasis, phase 3 (JAMA Oncol, PMID 42166134); high-dose vitamin D3 added to standard treatment in mCRC (SOLARIS, JAMA, PMID 42545685; no PFS benefit).
- **Rectal and GI radiochemotherapy:** total neoadjuvant therapy with long-course RT vs CRT (JCO, PMID 42492015); irinotecan added to preoperative CRT (ARISTOTLE, Lancet Oncol, PMID 42508427); adjuvant chemotherapy ± CRT in pancreatic head adenocarcinoma (JCO, PMID 42456089); adjuvant SOX vs S-1 in gastric cancer (CAPITAL, PMID 42600610); oral vs IV paclitaxel in gastric cancer (PMID 42617423); TACE + thermal ablation in HCC (TORCH, JAMA Oncol, PMID 42530948).
- **Breast:** anthracycline→taxane vs taxane-carboplatin (neo)adjuvant, phase 3 (Ann Oncol, PMID 42218963); oral paclitaxel DHP107 vs IV weekly paclitaxel (OPTIMAL, PMID 41833903); intensified vs standard chemotherapy then olaparib in HRD disease (PMID 42372741); eribulin vs taxane with dual HER2 blockade (PMID 42480197); fulvestrant vs capecitabine maintenance (PMID 42168151); neoadjuvant palbociclib + ET vs chemotherapy (PMID 41951647); ovarian reserve as a marker of adjuvant chemotherapy benefit (PMID 42613139).
- **Lung:** gotistobart or docetaxel in squamous NSCLC (PRESERVE-003 stage 1, Nat Med, PMID 41896648); LIBRETTO-431 chemoimmunotherapy comparator outcomes in RET-fusion lung cancer (PMID 42431264).
- **Head and neck / gynaecological:** weekly vs tri-weekly cisplatin CRT (PMID 41812622; JCOG1008 long-term, PMID 42361282); adjuvant metronomic capecitabine in nasopharyngeal carcinoma (Nat Cancer, PMID 42380630); induction TPF vs adjuvant PF

(PMID 42604670); relacorilant + nab-paclitaxel OS in platinum-resistant ovarian cancer (ROSELLA, Lancet, PMID 41974149); cisplatin CRT non-inferiority in cervical cancer (ConCERT, PMID 42636269); adjuvant 5-FU-platinum vs paclitaxel-platinum in penile cancer (PMID 42636270).

- **Hematology:** tafasitamab + lenalidomide + R-CHOP in high-risk DLBCL, phase 3 (Lancet, PMID 42217458); SWOG S0016 15-year follow-up (PMID 41746629); low-dose pediatric AML induction (PMID 41812214); CPX-351 vs standard induction in pediatric AML (AAML1831, JCO, PMID 42497367); DA-EPOCH-R + venetoclax (PMID 42660130); high-dose chemotherapy + ASCT vs non-myeloablative consolidation in PCNSL (Lancet, PMID 42486133).
- **Sarcoma and pediatric solid tumours:** Euro-EWING99 long-term (PMID 42537000); zoledronic acid added to Ewing consolidation (EE2012, PMID 41832299); trabectedin ± regional hyperthermia (PMID 41962307); ependymoma post-irradiation chemotherapy (PMID 41423745).
- **Pharmacology and dosing:** PK-guided melphalan (PMID 42654041); metronomic capecitabine combinations (PMID 41986320); paricalcitol added to gemcitabine/nab-paclitaxel in pancreatic cancer (Nat Cancer, PMID 42185478; safety run-in with paired biopsies showing stromal/CD8 modulation, filed here but really a TME study).

The clinical_other layer beneath this is large: real-world cohorts, secondary analyses of completed trials, and Chinese-language regimen comparisons. They are retained for completeness but should not be weighted with the randomised readouts.

2. Tumour-microenvironment and metabolic therapy

tme_metabolic is ~1,300 records, ~90% preclinical. Clinically it is thin and mostly combination arms inside IO trials (oleclumab/CD73 with durvalumab and SBRT, Nat Med, PMID 42350643; adenosine-axis and TGF- β trap agents in note 02/04). The preclinical literature is coherent around a few axes:

1. **Myeloid reprogramming:** CSF1R, CD40 agonism, TREM2, SIRP α /CD47, MARCO and macrophage-polarising nanoparticles; consistent across models, inconsistent in trials.
2. **Fibroblast/stroma:** FAP-directed radioligands and CAR-T (notes 01, 07); LRRC15, hyaluronidase and Hedgehog-pathway stroma modulation revisited with better patient selection.
3. **Metabolic checkpoints:** IDO/TDO revival via combination logic, glutaminase and glutamine antagonists (DRP-104-like prodrugs), arginase, lactate/MCT1-4, PI3K δ / γ myeloid inhibitors; ketogenic/fasting-mimicking diets as chemotherapy sensitisers with early randomised signals.
4. **Ferroptosis/cuproptosis inducers,** GPX4 inhibition and iron-loaded nanocarriers as a distinct sub-literature that overlaps nanomedicine_delivery.
5. **Neuro-immune and stress-axis modulation:** β -blockade (propranolol, PMID 42662047), angiotensin-pathway modulation (PMID 42081750), and sympathetic denervation.
6. **Angiogenesis** — anti-VEGF backbone combinations (bevacizumab with PD-1/VEGF bispecifics in note 02; ramucirumab, PMID 42081056) plus normalisation strategies to improve drug penetration.

3. Drug repurposing and chemoprevention

The repurposed bucket (~250 records) is the least evidence-dense. Notable clinical entries: aspirin for cancer prevention in Lynch syndrome dose comparison (CaPP3, Lancet Gastroenterol Hepatol, PMID 42425127); metformin's Lac-Phe axis in prostate cancer (PMID 41942753) and enzalutamide + metformin (PMID 41862335); eflornithine for gastric precancerous lesions (PMID 41801147); itraconazole for basal cell carcinoma (PMID 42101092); GLP-1 receptor agonist outcomes in breast cancer/diabetes (PMID 41801847); high-dose vitamin C pharmacokinetics (PMID 42378816) and toxicity mitigation (PMID 41973719); ivermectin/mebendazole prospective observational cohort (PMID 42203321) — hypothesis-generating at best; curcumin/omega-3 lung chemoprevention imaging endpoints (PMID 42649879).

Preclinically, statins, metformin, disulfiram, antipsychotics, antihelminthics, SGLT2 inhibitors, and antihistamines recur; the field's structural problem — supraphysiological in-vitro concentrations — is unchanged in this window. Rows in this bucket with `evidence = preclinical` should be treated as leads, not evidence.

4. Cross-cutting observations

- The chemotherapy backbone remains the control arm in the majority of positive phase 3 trials in other buckets (ADC, bispecific, checkpoint, RAS). Reading `chemo_conventional` together with those buckets gives the comparator context.
- The TME literature is increasingly combination-first: fewer monotherapy claims, more triplets with checkpoint blockade or radiation.
- Repurposing is trending toward prevention and interception (Lynch aspirin, KRAS vaccines in note 05, eflornithine) rather than late-line treatment.

10 — Methods, limitations, and caveats

1. Pipeline

harvest.py	Europe PMC REST search, FIRST_PDATE:[2026-03-09 TO 2026-09-09], SRC in {MED, PMC, PPR}; 67 compound queries across 20 modality groups, trial vocabulary, preclinical-model vocabulary, 58 high-impact venues, pediatric + CNS disease terms. Cursor pagination, 500/page,
	up to 16 pages per query. Output: raw_harvest.json (~99k unique records), harvest.log (per-query counts and overlap).
curate.py	Rule-based curation → index.tsv + curated.json.
fetch_fulltext.py	Europe PMC fullTextXML for prioritised PMCID-bearing rows → fulltext/.
build_pdf.py	REPORT.md + notes/ + grouped reference appendix → PDF.

No API key, no manual editing of the data files. Re-running the three data scripts regenerates everything; the only drift will come from Europe PMC indexing new items into the window (typically back-filled MEDLINE records) and from journals correcting metadata.

2. What the curation rules do

Applied in order to each harvested record:

1. **Publication-type and title exclusions** — errata, editorials, letters, protocols, case reports, guidelines, reviews (with the exception below), cost-effectiveness, bibliometrics, nursing/psychosocial, screening/epidemiology, prognostic-signature/bioinformatics, diagnostic imaging/radiomics, supportive-care and toxicity-management studies, surgical-technique comparisons, trial-methodology papers, veterinary and non-cancer disease contexts. ~42k records removed here.
2. **Cancer context** — a cancer term in the title, or ≥ 3 in the abstract. ~21k removed.
3. **Therapy context** — a therapy term in the title, or ≥ 4 in the abstract. ~7.6k removed.
4. **Evidence level** — explicit phase in title/abstract (phase 3 checked before generic "randomised"), then strong clinical markers (trial registry ID, enrolment/ORR/DLT/RP2D language), then observational human studies (`clinical_other`), meta-analyses/systematic reviews (`evidence_synthesis`), and preclinical models. Medicinal-chemistry papers that mention comparator agents' clinical phase are forced to preclinical. Records with no evidence signal are dropped.
5. **Observational and generic-modality tightening** — `clinical_other/` `evidence_synthesis` rows must have a therapy term in the title; `other_therapy` (catch-all modality) rows must name a concrete agent class in the title unless preclinical.
6. **Primary modality** — 26 buckets scored on title (weighted) + abstract; ties broken by a fixed priority order that favours the more specific bucket (e.g. `bispecific_tce` over `antibody_other`, `radiopharm` over `radiotherapy`).
7. **Disease tags, in-vivo flag, preprint flag** → `topics`.
8. **Venue tier** from a curated journal list after stripping Europe PMC suffixes (": official journal of...", "(London, England)", leading "The").

9. **Deduplication** on normalised title (preferring the MEDLINE record over the preprint, and the record with a PMID) and then on DOI (keeping the earlier-dated record).

3. Known weaknesses

- **Recall vs precision.** The pipeline was tuned by iterative spot-checking of high-visibility strata (tier 1-2, phase 1-3, other_therapy). Precision in clinical_other and tier-3 preclinical is lower; expect residual supportive-care, prognostic, and surgical papers, especially when a title contains "treatment" or "therapy" generically. Conversely, some genuine therapy papers were removed by broad exclusions (e.g. neurocognitive-protection RCTs during cranial RT, "late effects" trials, "reconstruction"-titled studies).
- **Phase assignment is regex-based.** "Phase 3" in an abstract that cites a phase 3 trial can promote a secondary analysis or a real-world cohort to clinical_phase3. Long-term follow-ups and subgroup analyses of completed trials are therefore over-represented in the phase buckets relative to primary readouts. Use the title to confirm.
- **Single primary modality.** Combination trials (e.g. ADC + checkpoint, RT + immunotherapy, kinase inhibitor + chemotherapy) are filed under one bucket; the other appears only as also:<bucket> in topics when it is in the title. Counting by category therefore undercounts combinations.
- **Tier is venue, not quality.** It is a triage aid. Preprints are tier 3 by construction.
- **Date is Europe PMC firstPublicationDate.** Online-first and issue dates can differ by months; a small number of records first indexed in the window were published earlier in print, and some March 2026 online-first papers are missing because their first-publication date predates the window.
- **Europe PMC coverage.** MEDLINE + PMC + indexed preprint servers (bioRxiv, medRxiv, Research Square, Preprints.org, Authorea, F1000). Not covered: conference abstracts (ASCO/ESMO/AACR/ASH 2026 abstracts are absent unless published as papers), Embase-only journals, Chinese-language journals not in MEDLINE, and grey literature/press releases. This biases the corpus toward journal publication of trials rather than first disclosure.
- **Abstract-only classification.** All 18.4k records were classified from title + abstract; ~9.7k lack a PMCID at all. The 648 full-text XML files cover only the phase 1-3 trial reports with a PMCID (811 eligible, 700 attempted under the file cap, 52 not open-access at Europe PMC). Tier 1-2 preclinical rows were eligible in principle but fell below the cap; raise MAX_FILES in fetch_fulltext.py to extend coverage.
- **No manual adjudication of every row.** 18k rows were not read individually. The notes cite only records whose title and abstract were checked; the appendix in the PDF omits tier-3 preclinical and tier-3 clinical_other rows.

4. Preprints

~1.3k records (7%) are preprints (src = PPR, tagged preprint). They are kept because for preclinical and some early clinical work the preprint is the only record in the window. They are not peer-reviewed, may never be, and several already have a journal version outside the window that the title-dedup could not match. Treat them as leads.

5. Safe use

- For "what changed clinically in the last six months": filter `evidence in {clinical_phase3, clinical_phase2}` and `tier in {1, 2}`; ~600 rows; read titles, then the cited notes.
- For "what is the preclinical field doing in modality X": filter `category = X`, `evidence = preclinical`, and sort by `tier`; skim titles; the `in_vivo` topic separates model-based work from in-vitro-only.
- Do not use the corpus for treatment decisions; it is a literature index, not a guideline, and it contains negative trials, retracted-later preprints, and low-quality cohorts by design.
- Verify any claim in the notes against the linked record before quoting it; the notes summarise abstracts, not full results, and outcome claims are only as good as the abstract wording.

Appendix: reference list

18424 curated records (2026-03-09 to 2026-09-09), grouped by modality and evidence level, ordered by priority then date. Tier-3 preclinical records and tier-3 'other clinical' records are counted rather than listed; all are in `index.tsv`.

checkpoint (2200)

Phase 3 / randomised (125)

- Colombo N, Zsiros E, Parma G, Rulli E, Sebastianelli A, Bidzinski M, Gallardo C, Matanes E, Hasegawa K, Kose F, et al. Pembrolizumab plus weekly paclitaxel in platinum-resistant recurrent ovarian cancer (ENGOT-ov65/KEYNOTE-B96): a multicentre, randomised, double-blind, phase 3 study. *Lancet* (London, England) 2026-04-10. PMID 41974150.
- Yang Y, Yen CJ, Pan J, Wang H, Qu S, Chen N, Chen X, Sun Y, He X, Hu C, Lin L, Wu Y, Yuan S, Chen C, Xu X, Ma X, et al. First-Line Tislelizumab Plus Chemotherapy for Recurrent or Metastatic Nasopharyngeal Cancer: Three-Year Follow-Up of the Phase 3 RATIONALE-309 Randomized Clinical Trial. *JAMA oncology* 2026-04-01. PMID 41746630.
- Xiong A, Yao W, Zheng W, Yu Y, Chen P, Zhong H, Ge J, Wang H, Chen B, Wang H, Fan Y, Yang Y, Pu X, Song X, Wang Q, et al. Sacituzumab tirumotecan plus pembrolizumab versus pembrolizumab in PD-L1-positive advanced non-small-cell lung cancer (OptiTROP-Lung05): interim analysis of a randomised, open-label, phase 3 trial. *Lancet* (London, England) 2026-05-29. PMID 42214392.
- Zhong H, Wang J, Yang R, Luo Y, Zuo W, Zhang W, Xie C, Li Q, Liu Q, Xu X, Wang Q, Yu Y, Chen Y, Yi T, Min X, Shi J, et al. Benmelstobart plus anlotinib versus pembrolizumab as first-line treatment for PD-L1-positive, advanced non-small-cell lung cancer (CAMPASS): a blinded, randomised, controlled, phase 3 trial. *The Lancet. Oncology* 2026-03-10. PMID 41825453.
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+ 447 tier-3 records not listed.

chemo conventional (2075)

Phase 3 / randomised (176)

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+ 857 tier-3 records not listed.

tme metabolic (1341)

Phase 3 / randomised (9)

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Phase 2 (5)

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+ 587 tier-3 records not listed.

rna gene therapy (737)

Phase 2 (2)

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+ 546 tier-3 records not listed.

cell therapy (734)

Phase 3 / randomised (6)

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epigenetic (444)

Phase 3 / randomised (1)

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repurposed (252)

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degrader (126)

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